

HazMat Incident - General

Initial Assessment

- Scene safety – enter scene only after cleared by IC, or treat patients in cold zone or warm zone with appropriate PPE
- What is suspected agent? Simple blast? Chemical agent? Nuclear weapon? Biologic agent?
- Multiple casualties? Severity?
- Are first responders affected?
- Triage mass casualties

Nuclear / Radioactive Agents

- Blast injuries
- Thermal burns
- Radiation Injuries
 - Nausea / vomiting
 - Weakness / fatigue
 - CNS effects / seizures

***Remember to use available dosimeter equipment

Chemical Agents

- **Nerve Agents**
 - Sarin
 - Soman
 - Tabun
 - VX
- **Pulmonary Agents**
 - Phosgene
 - Chlorine
 - Hydrogen sulfide
- **Vesicants**
 - Mustard
 - Lewisite
- **Cyanide Agents**

Biologic Agents

***Most biologic agents will cause minimal to no symptoms early after exposure. Treatment is supportive. Transport is mandatory.

Later symptoms:

- **Anthrax**
 - Inhalational – cough, fever, N/V, chest pain, cyanosis, respiratory distress, shock
 - Gastrointestinal – ulcers, N/V, bloody diarrhea, abdominal pain, sepsis
 - Cutaneous – ulcers, skin lesions, sepsis, shock
- **Plague** – fever, chills, dyspnea, resp distress, N/V, shock, bleeding, gangrene
- **Tularemia** – skin ulcer, fever, cough, resp distress
- **Brucellosis** – fever, weakness, cough, chills, cardiac infection, N/V, diarrhea
- **Q fever** – fever, chills, headache, weakness, cardiac infections, N/V, diarrhea
- **Smallpox** – fever, headache, delirium, rash (begins in mouth, spreads to face and then body)
- **Hemorrhagic Fever** – fever, subconjunctival (eye) bleeding, N/V (bloody), shock

Food-borne Illness – more likely to show signs and symptoms early (nausea, vomiting, diarrhea, dehydration)

- **Botulism** – N/V, diarrhea, paralysis within several hours of exposure

EMT/AEMT

Scene Safety

- Do not enter scene until deemed safe to do so by Incident Commander / HazMat Team
- Always wear appropriate PPE!
- Decontaminate patient as appropriate
- Follow General Medical/Trauma Care Guideline

Paramedic

Chemical – Treat per CBRNE / HazMat Incident – Refer to Handtevy Toxicology and WMD Drugs / Emergency Response Guidebook / Poison Control

Radiologic / Nuclear / Biologic

- Get as far away from incident as possible and shield crew and patient (Time, Distance, Shielding)
- **NS/LR** 500-1000mL IV / IO bolus to maintain SBP > 90 and normal mental status
 - If patient is still hypotensive (SBP ≤ 90), continue fluids
- Treat N/V per Guideline
- Treat Seizure per Guideline